

1. Administrative & Study Identification

Full Study Title: Clinical Study To Further Evaluate The Efficacy Of Dabrafenib Plus Trametinib In Patients With Rare BRAF V600E Mutation-Positive Unresectable or Metastatic Solid Tumors **Short Title/Acronym:** Efficacy of Dabrafenib + Trametinib in Rare BRAF V600E Mutated Solid Tumors **Protocol Number:** CDRB436IIC01 **NCT Number:** NCT05868629 **Sponsor Name:** Novartis **Investigational Product:** Dabrafenib plus Trametinib (commercial non-investigational formulation) **Phase of Development:** Phase IV **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and clinical benefit of dabrafenib plus trametinib in pediatric and adult patients with rare BRAF V600E mutation-positive unresectable or metastatic solid tumors. **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Duration of Response (DOR), Overall Survival (OS), and overall safety/tolerability of the drug combination in this specific rare patient population.

2.2 Background & Rationale While the combination of dabrafenib and trametinib has established efficacy and regulatory approval across several common BRAF V600E-mutated malignancies, there is a medical necessity to further capture real-world efficacy and safety data in patients with *rare* solid tumors harboring this mutation. This pragmatic study evaluates real-world clinical outcomes for patients in whom a clinical decision has already been made to use this combination therapy, providing crucial data for these under-represented rare oncology indications.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Pragmatic, Prospective, Open-label) **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 40 **Number of Sites:** Multicenter **Estimated Study Start:** 06-Feb-2024 **Estimated Primary Completion:** 03-Mar-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years (Pediatric, Adult, and Older Adult patients). 2. Diagnosed with rare BRAF V600E mutation-positive unresectable or metastatic solid tumor confirmed by a local laboratory test. 3. At least 1 measurable lesion as defined by RECIST v1.1 per local review. 4. Patient for whom a decision has already been made to be treated with dabrafenib and trametinib irrespective of trial participation.

4.2 Exclusion Criteria 1. Diagnosis of melanoma, Non-Small Cell Lung Cancer (NSCLC), Colorectal Cancer, Anaplastic Thyroid Cancer, Biliary Tract Cancer, or glioma. 2. Previously treated with dabrafenib and/or trametinib for the current disease, or having received the therapy within the past year for another malignancy. 3. Any unstable concurrent medical disorder that can impact participant safety or study outcomes.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Commercial dabrafenib plus trametinib administered according to local guidance and product labeling. **Route of Administration:** Oral (Solid capsules/tablets or liquid formulation, if approved and commercially available locally). **Duration of Treatment:** Continuous until disease progression, unacceptable toxicity, or death.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures and medications as determined by the treating physician for symptom management. **Prohibited:** Concurrent administration of other investigational therapies or antineoplastic agents that conflict with BRAF/MEK inhibition.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, BRAF V600E Mutation Confirmation, Baseline Imaging (RECIST v1.1)
Baseline	Day 0	Treatment Initiation, Baseline Safety Labs
Interim	Every 8-12 Weeks	Tumor Response Assessments (Scans), Safety Labs, Efficacy Assessment
End of Study	PD or Completion	Final Vital Signs, Study Exit, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Proportion of study participants with a best overall response of Complete Response (CR), Partial Response (PR), or Stable Disease (SD) lasting for a duration of

at least 24 weeks. **Measurement Method:** Local or independent central review based on RECIST v1.1 or other relevant response criteria.

7.2 Secondary & Exploratory Endpoints * Progression-Free Survival (PFS): Time from tumor response onset to disease progression or death. * Duration of Response (DOR). * Overall Survival (OS).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous evaluation during clinical visits through physician assessment and patient reporting. **Serious Adverse Events (SAEs):** Reported by the clinical site within 24 hours of site awareness to the sponsor's safety department. **Data Monitoring Committee (DMC):** Internal sponsor review committee to monitor cumulative safety data for the pediatric and adult cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy assessments; Safety Set for all participants who received at least one dose of the medication. **Sample Size Justification:** A target of $N=40$ patients provides sufficient descriptive power to establish clinical benefit parameters in a rare, mutation-driven oncology subset. **Handling of Missing Data:** Standard survival analysis methodologies will be applied, including censoring for time-to-event endpoints (PFS, OS) at the date of the last adequate tumor assessment.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine risk-based monitoring of source data verification to ensure RECIST v1.1 adherence and accurate adverse event reporting. **Audit Trail:** Secure, compliant EDC (Electronic Data Capture) systems with continuous tracking and automated data clarification forms.

11. Study Identifiers & Governance

Primary ID: CDRB436IIC01 **Registry Number:** NCT05868629 **Sponsor/Lead Organization:** Novartis **Study Title:** Clinical Study To Further Evaluate The Efficacy Of Dabrafenib Plus Trametinib In Patients With Rare BRAF V600E Mutation-Positive Unresectable or Metastatic Solid Tumors **Official Regulatory Title:** Phase IV Pragmatic Single-arm Prospective, Open Label Study in Pediatric

12. Clinical Framework (Oncology Specific)

Primary Condition: Rare Unresectable or Metastatic Solid Tumors
Diagnosis Threshold: Confirmed BRAF V600E mutation by local laboratory test
Medical Methodology: Targeted Kinase Inhibition Therapy (BRAF/MEK inhibitors)
Optimization Target: Best Overall Response (CR/PR/SD) lasting $\geq$ 24 weeks

13. Study Procedures & Methodology

Management Pathway: Pragmatic clinical management using commercial dabrafenib plus trametinib based on the existing standard of care
Education Protocol (HCP): RECIST v1.1 application guidelines and management of targeted therapy-related adverse events
Education Protocol (Patient): Self-reporting of adverse events and guidance on oral medication adherence
Quality Audit Mechanism: Independent central review of imaging for tumor response verification

14. Observation & Follow-Up Schedule

Total Duration: From enrollment until disease progression, unacceptable toxicity, or primary completion (March 2028)
Onsite Visit Cadence: Guided by local standard practice (typically every 8-12 weeks synchronized with tumor imaging)
Remote Monitoring Frequency: Continuous electronic monitoring of entered case report forms
Checklist Review Interval: Routine review at each clinical response assessment milestone

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				